Surgical Prophylaxis

Surgical Prophylaxis for Transplant

Principles of surgical antimicrobial prophylaxis for transplant:

1. If patients known to be colonized with multidrug resistant organism, to tailor according to individual susceptibilities
2. If patient is being treated for an active infection at the time of organ transplantation, the antibiotic regimen should be altered to target specific pathogens based on the current infection and individual risk factors

CHAPTER 4.13	SURGICAL PROPHYLAXIS		
Procedures	**Suggested Regimen**		**Notes**
	Preferred	**Alternative**	
Transplant			
Liver Common pathogens causing SSI: Gram-negative organisms (predominantly	Cefoperazone 50 mg/kg/dose IV single dose then q8H for 1-2 days +		• Universal antifungal prophylaxis is probably not necessary since the risk of invasive candidiasis is low in uncomplicated cases. • Antifungal prophylaxis is generally reserved for patients with two or more of the following risk factors:

CHAPTER 4.13	SURGICAL PROPHYLAXIS		
Procedures	**Suggested Regimen**		**Notes**
	Preferred	**Alternative**	
Enterobacteriaceae, *Acinetobacter* species, and more rarely *Pseudomonas*); there are also high rates of infection with *Enterococci*, *S aureus*, coagulase-negative *Staphylococcus* spp, and *Candida* species.	Ampicillin 25 mg/kg/dose IV single dose then q6H for 1-2 days ± Metronidazole 7.5 mg/kg/dose IV single dose		■ Need for reoperation, ■ Re-transplantation, ■ Renal failure, ■ Known colonization with *Candida* species. • Risk of fungal infection is also increased with prolonged initial procedure or transfusion of >40 units of cellular blood products, but this cannot be predicted before the procedure. • Choice of antifungals (ONE of the following): ■ Micafungin 1 mg/kg IV single dose ■ Fluconazole 6 mg/kg IV single dose • If high risk for invasive fungal infection, duration depends on the individual risk. • To consider Piperacillin/Tazobactam if the suspicion for *Pseudomonas* infection is high.